NCCP 00311 · VERSION 4

Your treatment, explained

Relapsed small-cell lung cancer · NCCP 00311 v4 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Topotecan by infusion · small-cell lung cancer

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your treatment course
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- My treatment
- My schedule
- Between treatments
- When to call
- Tests and review
- Supportive medicines
- Understanding the evidence
- Questions to ask

Sources

UNDERSTAND YOUR TREATMENT

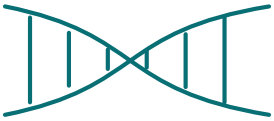
Your chemotherapy

This treatment aims to slow or shrink small-cell lung cancer that has returned after treatment, when repeating the first treatment is not appropriate. Benefit varies; your team explains the aim for you.

TOPOISOMERASE-I INHIBITOR CHEMOTHERAPY

Topotecan infusion

Hycamtin; supplied brand may vary



Disrupts DNA copying and repair

Topotecan blocks topoisomerase I, an enzyme that relieves strain while DNA is copied. Unrepaired DNA breaks can stop dividing cancer cells; healthy blood-forming and gut cells can also be affected.

Medicine reaches cells → DNA damage persists → Cell division disrupted

What to watch for

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Your treatment course

UP TO 6 CYCLES

Chemotherapy course

1 2 3 4 5 6

Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.

INDIVIDUAL PLAN

Review and next plan

Review

Your team reviews benefit and side effects and agrees whether to continue, change or stop.

Relative educational plan; your team confirms actual appointments and changes.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Check	Before treatment	During the cycle	Before next course	After treatment
Safety blood tests	Blood count, kidney and liver profile	Report symptoms; extra tests if needed	Blood count, kidney and liver review	Follow abnormal results as advised
Disease assessment	Confirm treatment aim	Symptoms reviewed	Scans or other tests on consultant plan	Agree next treatment or follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00311 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Topotecan by infusion · small-cell lung cancer: treatment and checks

Relapsed small-cell lung cancer

Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between courses	Course review
Treatment NCCP 00311	Confirm the exact treatment plan and contact numbers before starting.	Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.	Follow support plan and report symptoms	Review benefit/toxicity; next plan
Safety tests NCCP 00311	Blood count, kidney/liver tests	Review symptoms; extra tests if needed	Blood count, kidney and liver tests before treatment and before each cycle. Scans and follow-up follow the consultant plan.	Follow abnormal results

What each check is for

1

Before each course

Before starting

Safety and plan may change

Ask / act: Team confirms next treatment

Sources: NCCP 00311

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

YOUR CYCLE, DAY BY DAY

Your 21-day cycle.

Select a day to explore what to watch for. These are possible patterns, not a promise about when symptoms start or stop.

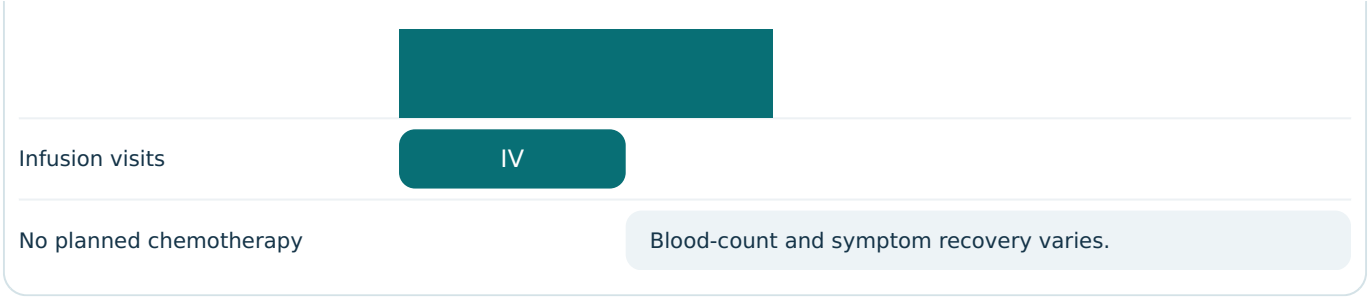
> The full information is available below. Interactive day, phase and date controls require JavaScript; use “Read the whole cycle as text” for the complete timeline.

Your date stays on this page only. It is not saved or added to a QR code.

Medicines on Day 1: Topotecan infusion

On a small screen, swipe the chart sideways. Each day is a keyboard-accessible button.





Effects vary; no risk curve or guaranteed recovery day is shown.

DAY 1

Treatment days

Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.

Your next step: Follow the confirmed prescription; report problems promptly.

The next cycle starts 21 days after this one, if your team confirms it.

> Do not use this page to calculate or change doses.

Read the whole cycle as text

Day 1–5: Treatment days

Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable. Follow the confirmed prescription; report problems promptly.

Day 6–21: Between treatments

No planned chemotherapy; continue prescribed support and monitoring. Do not restart until the team confirms the next cycle.

Sources: nccp

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before treatment

Confirm the exact treatment plan and contact numbers before starting.



STEP 2

Treatment visit or home course

Each infusion takes about 30 minutes. Blood review, preparation and observation add to appointment time. The consultant may prescribe only 3 or 4 treatment days in selected circumstances; follow the confirmed appointment plan.



STEP 3

Bring to review

Bring your medicine list, dose record and questions; report new symptoms before starting another course.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00311 v4 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Sickness and bowel symptoms

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

2 Blood tests and review

Blood count, kidney and liver tests are checked before treatment and before each cycle. Extra tests depend on symptoms and results.

3 Pregnancy, fertility and medicines

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

4 During the infusion

Tell the nurse immediately about rash, flushing, breathing difficulty, dizziness, or pain/swelling at the drip site.

Sources: nccp · smpc · ncis

My treatment

This treatment aims to slow or shrink small-cell lung cancer that has returned after treatment, when repeating the first treatment is not appropriate. Benefit varies; your team explains the aim for you.

How it works

Topotecan blocks topoisomerase I, an enzyme that relieves strain while DNA is copied. Unrepaired DNA breaks can stop dividing cancer cells; healthy blood-forming and gut cells can also be affected.

Sources: nccp · smpc

Evidence and limits

An older randomized trial found similar survival with intravenous topotecan and CAV chemotherapy, with improvement in several symptoms in the topotecan group. These historical groups do not predict an individual outcome or compare every treatment available today. Treatment choice also depends on previous response, time since treatment, fitness and preferences.

Sources: trial

TREATMENT REFERENCE · 02

My schedule

Topotecan is given by infusion on Days 1–5 of a 21-day cycle. Up to 6 cycles are planned, with earlier changes if treatment stops helping or side effects become unacceptable.

Administration

Each infusion takes about 30 minutes. Blood review, preparation and observation add to appointment time. The consultant may prescribe only 3 or 4 treatment days in selected circumstances; follow the confirmed appointment plan.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

Between treatments

Keep your prescription and contact plan to hand.

Sickness and bowel symptoms

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

Sources: nccp · smpc

Blood tests and review

Blood count, kidney and liver tests are checked before treatment and before each cycle. Extra tests depend on symptoms and results.

Sources: nccp · smpc

Pregnancy, fertility and medicines

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

Sources: nccp · smpc

During the infusion

Tell the nurse immediately about rash, flushing, breathing difficulty, dizziness, or pain/swelling at the drip site.

Sources: nccp · smpc

TREATMENT REFERENCE · 04

When to call

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Blood counts

An infection can be serious even after treatment days have finished. A normal earlier blood test does not rule out a new problem.

Sources: nccp · smpc

TREATMENT REFERENCE · 05

Tests and review

Blood count, kidney and liver tests before treatment and before each cycle. Scans and follow-up follow the consultant plan.

Scans and tumour markers

Your consultant decides which are relevant and when. Do not assume a fixed imaging interval or that tumour markers are required for everyone.

Sources: nccp · smpc

After treatment

Agree follow-up, symptom support and the next plan before the course ends.

Sources: nccp · smpc

Why this treatment may be offered

An older randomized trial found similar survival with intravenous topotecan and CAV chemotherapy, with improvement in several symptoms in the topotecan group.

These historical groups do not predict an individual outcome or compare every treatment available today. Treatment choice also depends on previous response, time since treatment, fitness and preferences.

Sources: trial · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

Why can treatment be delayed?

Blood results or symptoms may make a delay or dose change safer. Your team confirms when to restart.

Are capsules gentler than an infusion?

Route alone does not predict treatment risk. Your team explains the relevant alternatives.

When will I have scans?

Your consultant sets the timing for your disease and treatment plan. No universal scan or tumour-marker interval is specified here.

Try explaining the plan back: Explain which days you take treatment, when your next blood tests are due and whom you will call tonight if unwell.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00311 v4. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00311 v4 · official regimen
- EMA Hycamtin product information
- NCCP NCIS medical oncology antiemetic guidance V8
- von Pawel 1999 randomized IV topotecan versus CAV

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6714/311_v4_Topotecan_Monotherapy_5day.pdf

https://www.ema.europa.eu/en/documents/product-information/hycamtin-epar-product-information_en.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/10080612/